

Lev Klarnet

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Education	Harvard University Ph.D., Business Economics	2023-2029 (expected)
	George Washington University B.A., Economics and International Affairs	2015-2019
Publications in Economics	“Is There Too Little Antitrust Enforcement in the US Hospital Sector?” (with Zarek Brot-Goldberg, Zack Cooper, and Stuart Craig), <i>American Economic Review: Insights</i>, 2024, 6(4), 526–542 From 2002 to 2020, there were over 1,000 mergers of US hospitals. During this period, the Federal Trade Commission (FTC) took enforcement actions against 13 transactions. However, using the FTC’s standard screening tools, we find that 20% of these mergers could have been predicted to meaningfully lessen competition. We then show that, from 2010 to 2015, predictably anticompetitive mergers resulted in price increases over 5%. We estimate that approximately half of predictably anticompetitive mergers had to be reported to the FTC per the Hart-Scott-Rodino Act. We conclude that there appears to be underenforcement of antitrust laws in the hospital sector.	
Publications in Health Policy	“Scanning the Health Care System in the US” (with David Cutler), <i>JAMA Health Forum</i>, 2026, 7(4) “Sale of Private Equity–Owned Physician Practices and Physician Turnover” (with Victoria Berquist and Leemore Dafny), <i>JAMA Health Forum</i>, 2025, 6(2)	
Working Papers	“Has the United States Bent the Health Care Cost Curve?” (with David Cutler), <i>Brookings Papers on Economic Activity</i>, 2026 In 2024, medical spending as a share of GDP was 15% below forecasts made in 2010 and only marginally higher than in 2010. Relative to expectations, the savings were nearly \$1 trillion in 2024. In light of this prolonged period of slower growth, we ask the question: has the United States bent the health care cost curve? We start with a model of medical spending that incorporates both the development of new technologies and the equilibrium use of those technologies. Technology development and incentives around its use may add to or subtract from spending growth. We then examine the drivers of the spending slowdown empirically. We attribute slower medical spending growth to five factors: the natural development of technologies that simultaneously improve health and lower cost; long-run supply elasticities that exceed short-run elasticities; improvements in population health; reimbursement changes that reduce demand and make demand more price-elastic; and reductions in the rate of price increases, likely driven by some of these same demand factors. Because these cost slowing mechanisms are expanding over time, we conclude that the United States has bent the health care cost curve, though not as much as it could be or will need to be bent. “Who Pays For Rising Health Care Prices? Evidence from Hospital Mergers” (with Zarek Brot-Goldberg, Zack Cooper, Stuart Craig, Ithai Lurie, and Corbin Miller),	

NBER Working Paper, 2024. *Revisions requested at American Economic Review.*

We analyze the economic consequences of rising health care prices in the US. Using exposure to price increases caused by horizontal hospital mergers as an instrument, we show that rising prices raise the cost of labor by increasing employer-sponsored health insurance premiums. A 1% increase in health care prices lowers both payroll and employment at firms outside the health sector by approximately 0.4%. At the county level, a 1% increase in health care prices reduces per capita labor income by 0.27%, increases flows into unemployment by approximately 0.1 percentage points (1%), lowers federal income tax receipts by 0.4%, and increases unemployment insurance payments by 2.5%. The increases in unemployment we observe are concentrated among workers earning between \$20,000 and \$100,000 annually. Finally, we estimate that a 1% increase in health care prices leads to a 1 per 100,000 population (2.7%) increase in deaths from suicides and overdoses. This implies that approximately 1 in 140 of the individuals who become fully separated from the labor market after health care prices increase die from a suicide or drug overdose.

**Honors and
Awards**

Hsieh Prize for the Best Honors Thesis in Economics, George Washington University
Undergraduate Research Award, George Washington University

**Predoctoral
Positions**

Research Assistant, Yale University, Professor Zack Cooper	2021-2023
Consultant, Bates White Economic Consulting	2020-2021